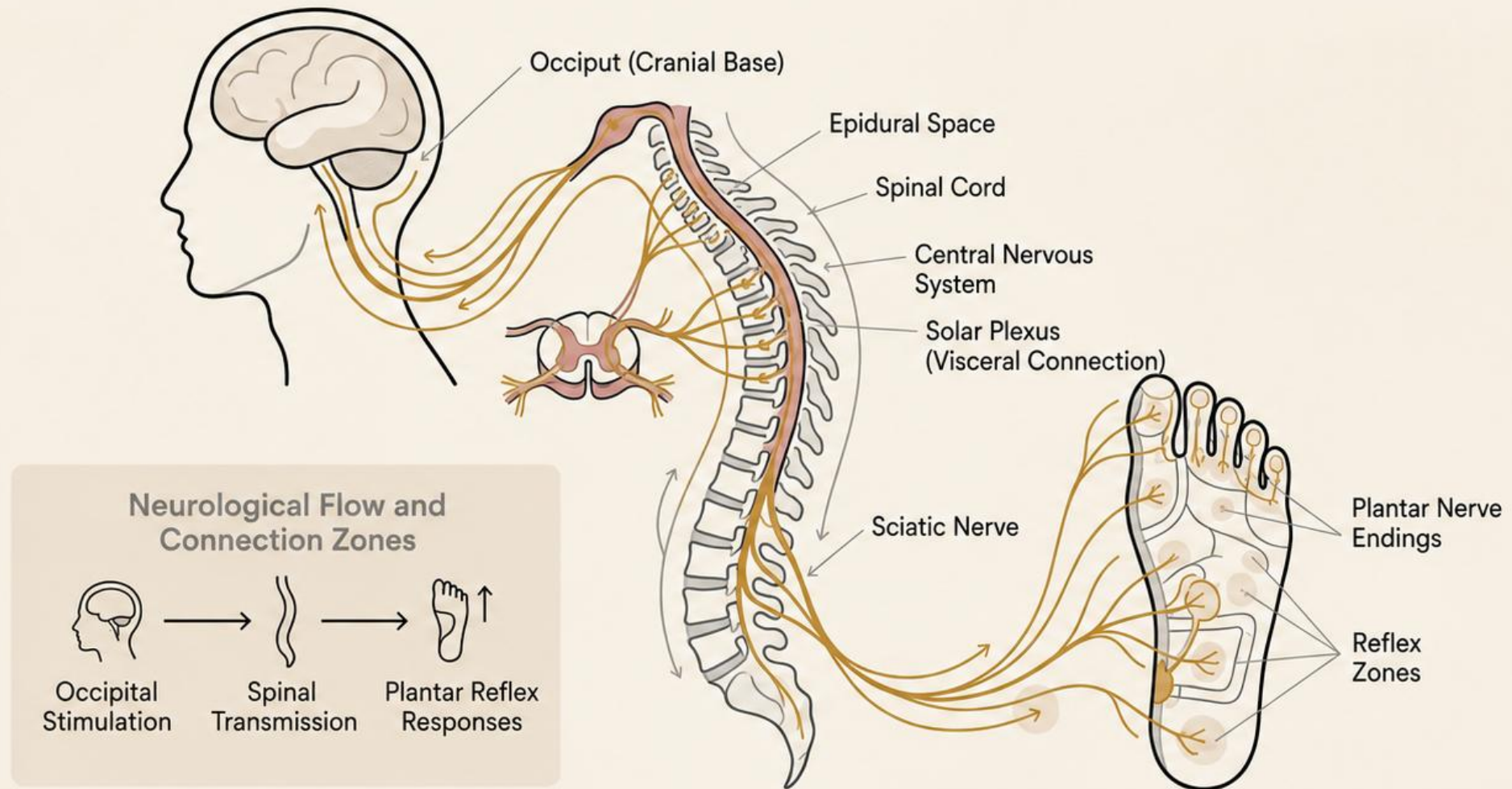


Occipito-Podal Reflexotherapy Treatment (ORT)

Chapter 2 — Principles, Mapping and Clinical Application



The Precision of the Therapeutic Gesture

Proximal interphalangeal
joint of the index finger

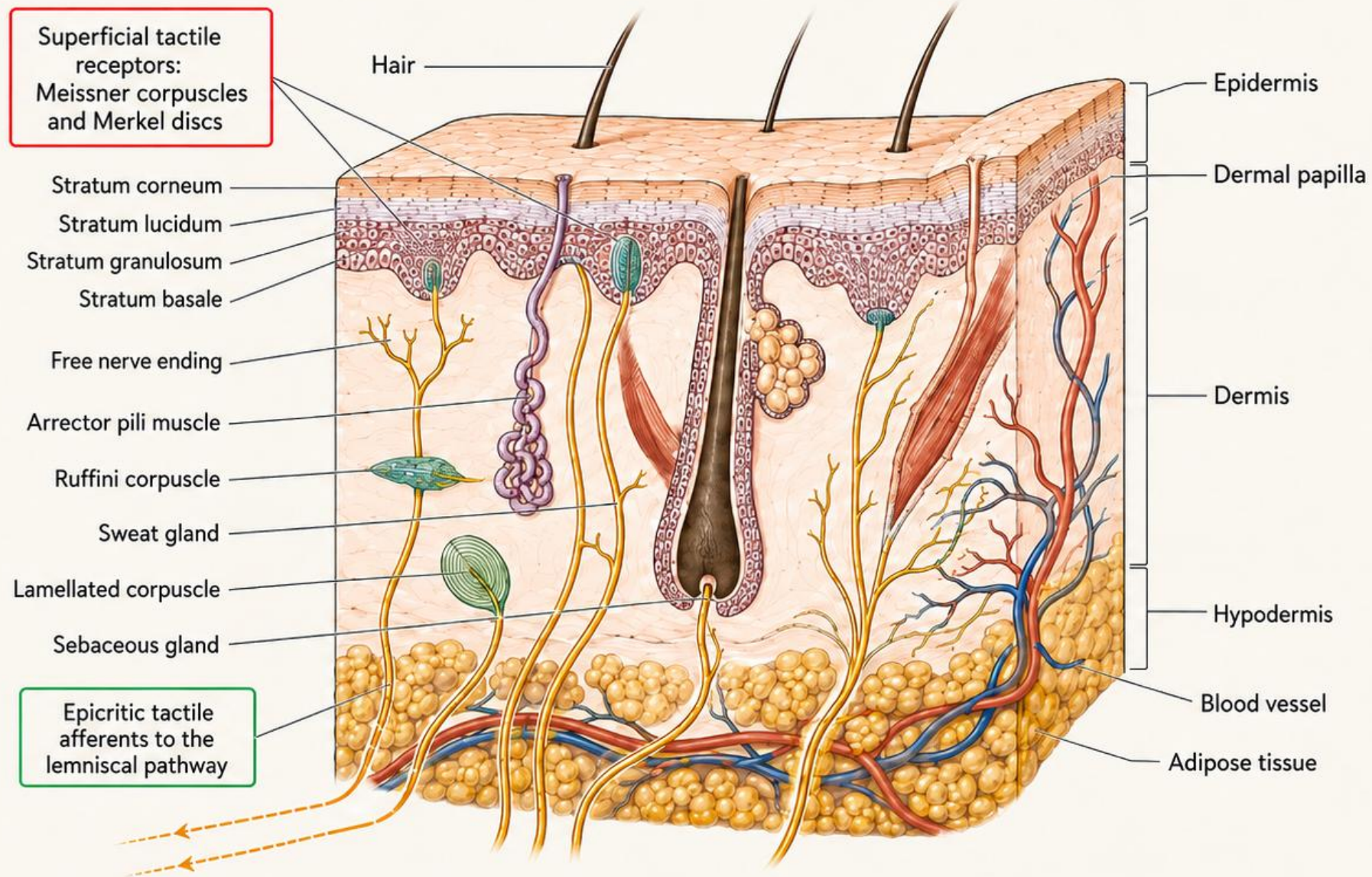
Thumb pad

An exclusively manual approach,
without the use of oil or cream,
based on the precision of
epicritic pressure.



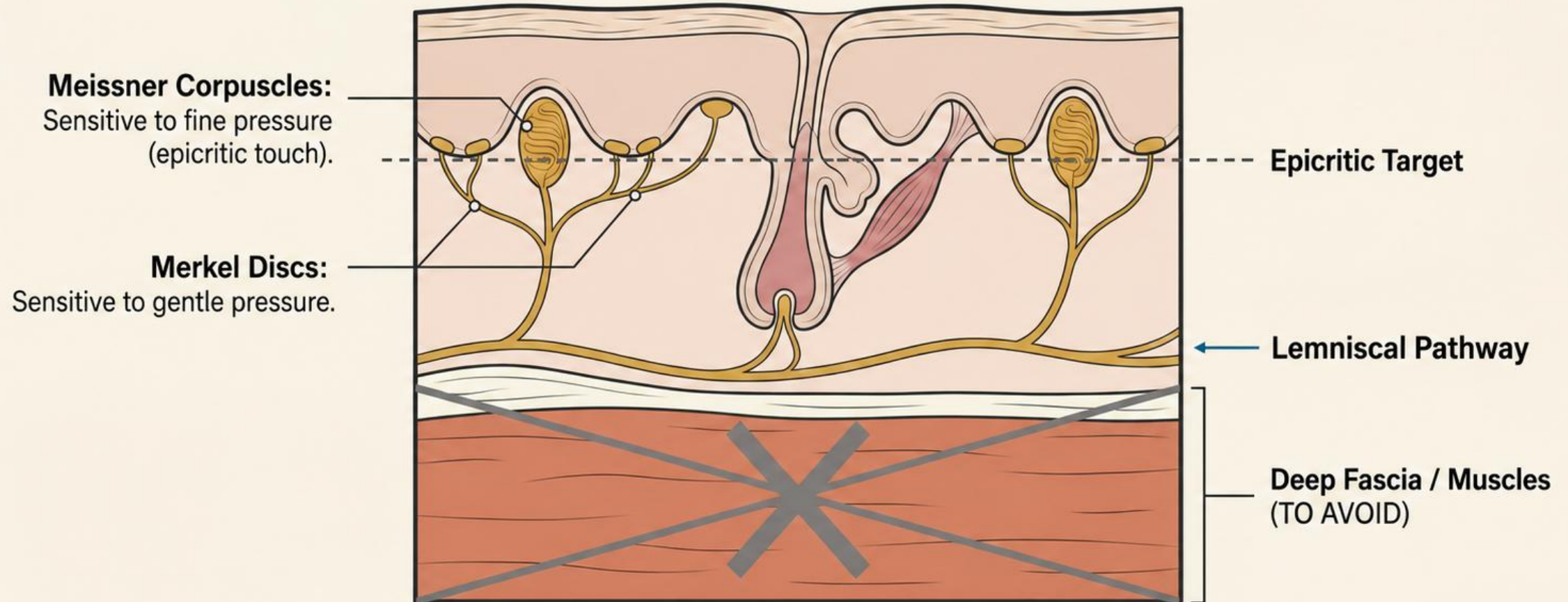
Force and kneading are contraindicated. The greater the pressure, the more difficult it becomes to read the bony landmarks and reflex zones.

The skin



Schematic representation of the layers of the skin, its appendages, and its main superficial tactile receptors.

Tissue Target: Epidermo-Dermal Layer



The objective is deformation of the skin, identified by the "drum-skin" sensation.
The greater the pressure, the more the perception of bony landmarks disappears.

The Neurological Vector

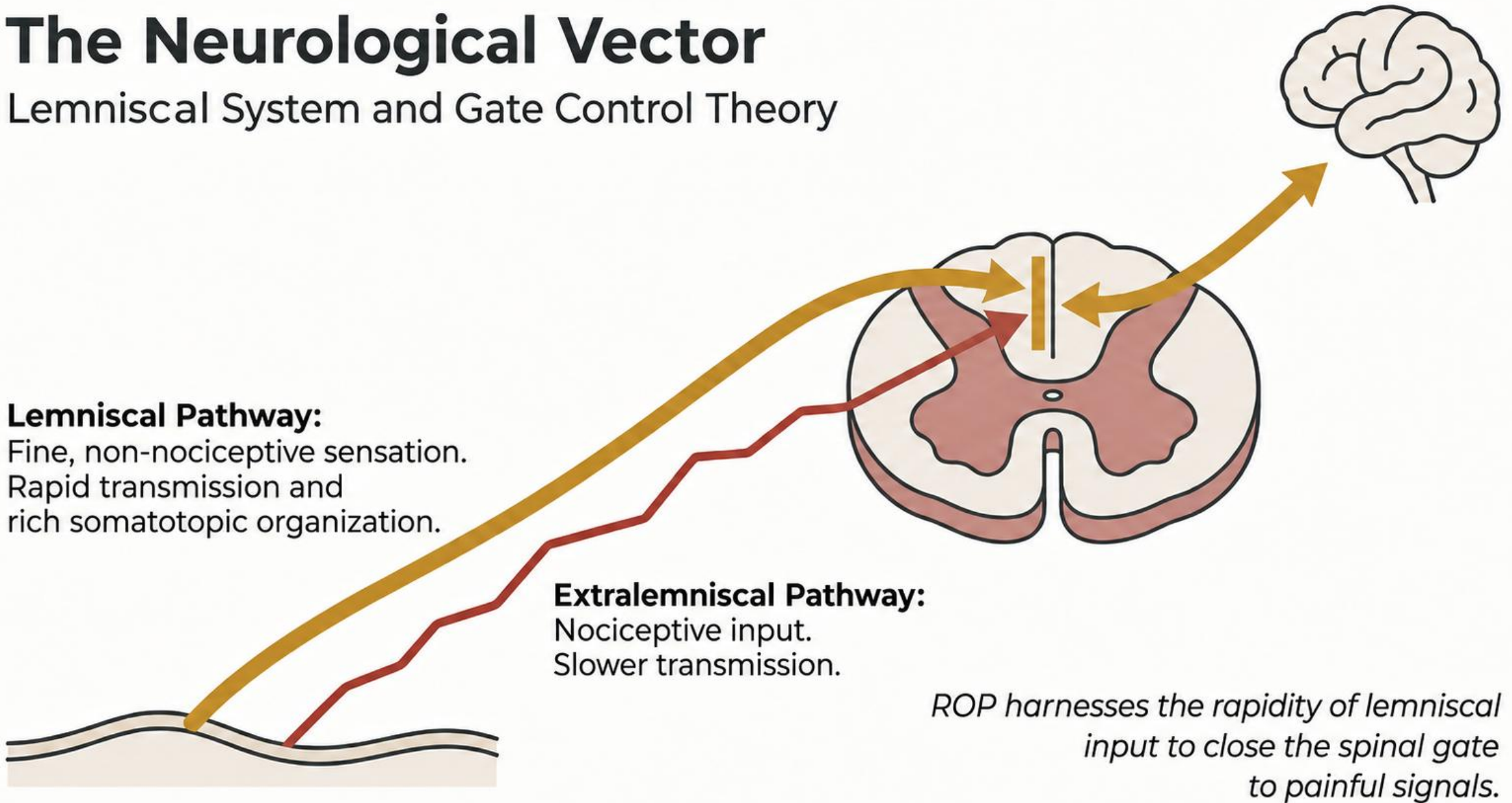
Lemniscal System and Gate Control Theory

Lemniscal Pathway:

Fine, non-nociceptive sensation.
Rapid transmission and
rich somatotopic organization.

Extralemniscal Pathway:

Nociceptive input.
Slower transmission.



*ROP harnesses the rapidity of lemniscal
input to close the spinal gate
to painful signals.*

Clinical Chronology: The Three Phases of Massage



Textural Diagnostic Phase

Action: Identification of roughness, densifications, and loss of glide.

Patient perception: Characteristic sensation of small pinpricks.



Therapeutic Phase

Action: Fine epicritic pressure. Mobilization of the epidermal-dermal layer.

Objective: Stimulate mechanoreceptors (without sharp pain, which indicates overstimulation).



'Watchful Waiting' Phase

Action: Post-stimulation latency period. Do nothing.

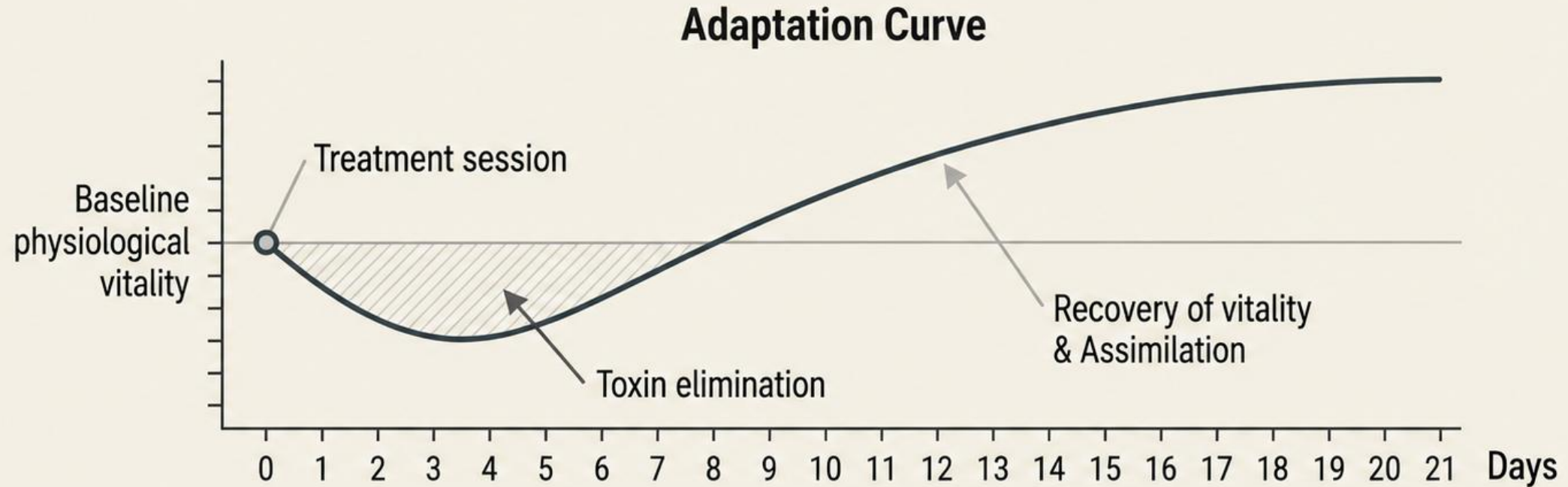
Objective: Allow the body to integrate neurovegetative adaptations.

Result: Restored suppleness.



The Neurovascular Assimilation Window

Why is a 3-week interval required?



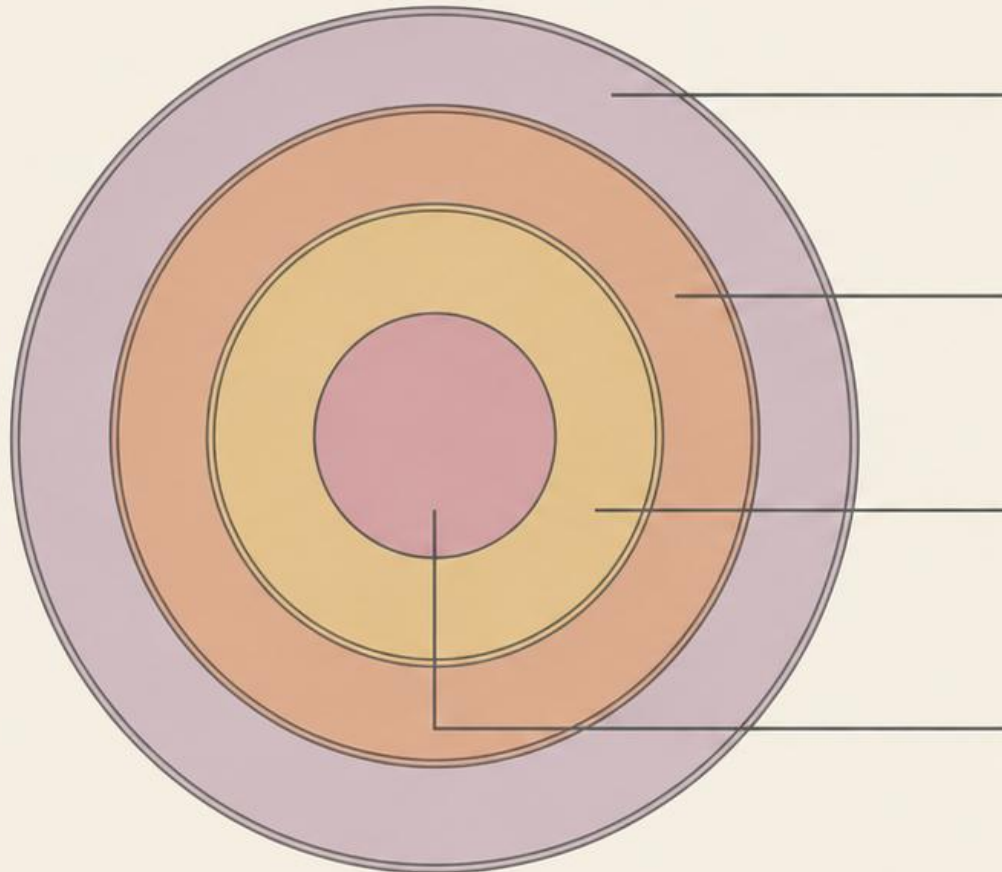
24 to 48 hours: Possible transient reactions (myalgia, fatigue, vivid dreams). This reflects the response to positive stress and toxin elimination.

Subsequent weeks: Adaptation of the immune, hormonal, and psychological systems.

Treatment sessions scheduled too close together do not allow sufficient time for neurovascular and structural processes to restore physiology in the viscera. Three sessions are generally sufficient.

Hierarchy of the Treatment Plan

There is no immutable protocol in a holistic approach.



1. Emergencies & Sequelae: Post-traumatic conditions, surgery, infection (immediate priority).

2. Dominant Fixations: Identified through the anamnesis and global/local listening tests.

3. Psycho-Emotional Sphere: Repressed past traumas (amygdala / hippocampus).

4. General Context (SGA): The complete history of the individual's adaptive processes.

Intervention Mapping

The Three Occipito-Podal Reflex Spheres

1. General Adaptation Syndrome (GAS)

Occiput-C1-C2, vagus nerve (X), falx cerebri, primary respiratory mechanism (PRM), venous system.

Objective: Support overall autonomic regulation.



3. Limbic System

Amygdala, hippocampus, insula. Brain-viscera balance.

Objective: Integrate the stress dimension and body memory.

2. Locoregional Syndrome

Prevertebral plexus, somatic plexuses, abdominal and pelvic cavities.

Objective: Treat viscerosomatic loops.

Anatomical Targets I: General Adaptation Syndrome

Support autonomic regulation before treating the local tissue.

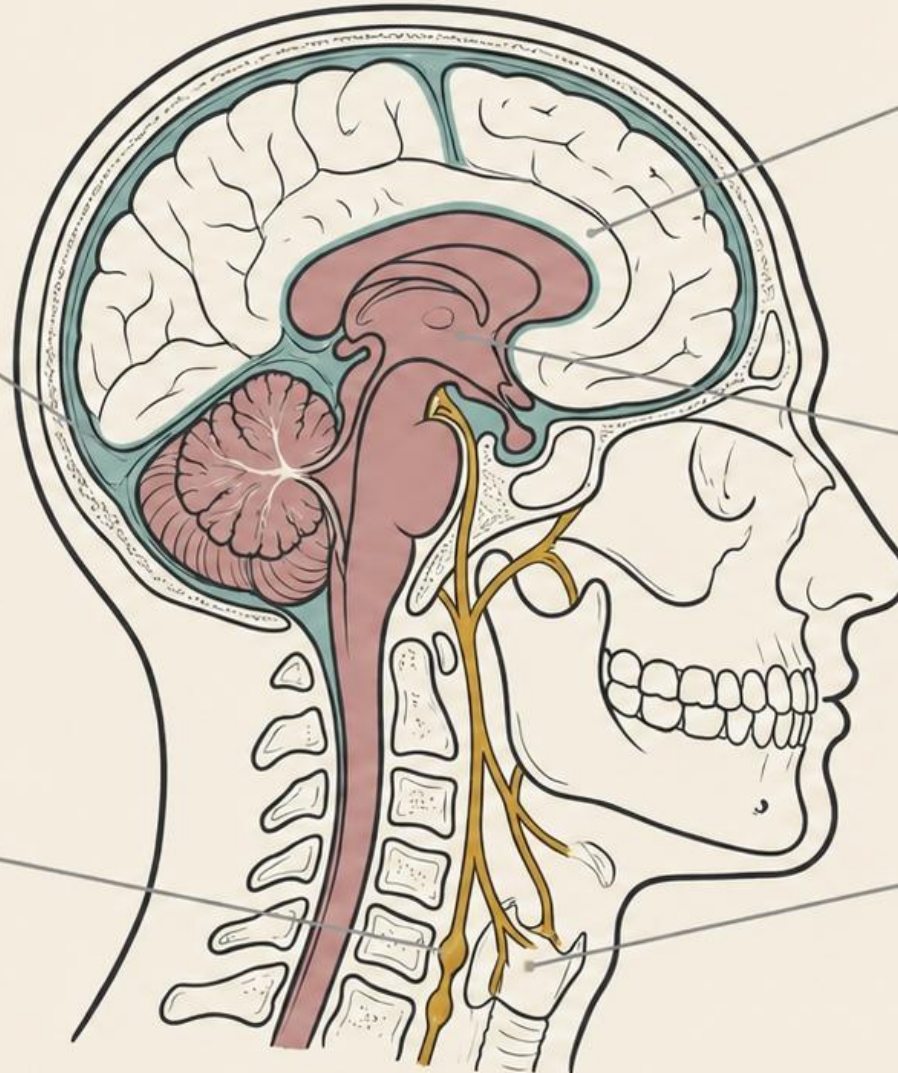
PRM (Primary Respiratory Mechanism): Occiput-C1-C2, cranial nerves V, X, XII, falx cerebri.

Cerebrospinal Fluid: Venous system, compression of the 4th ventricle, SSB-S2 synchronization.

Higher Centers: Diencephalon, brainstem, pituitary gland.

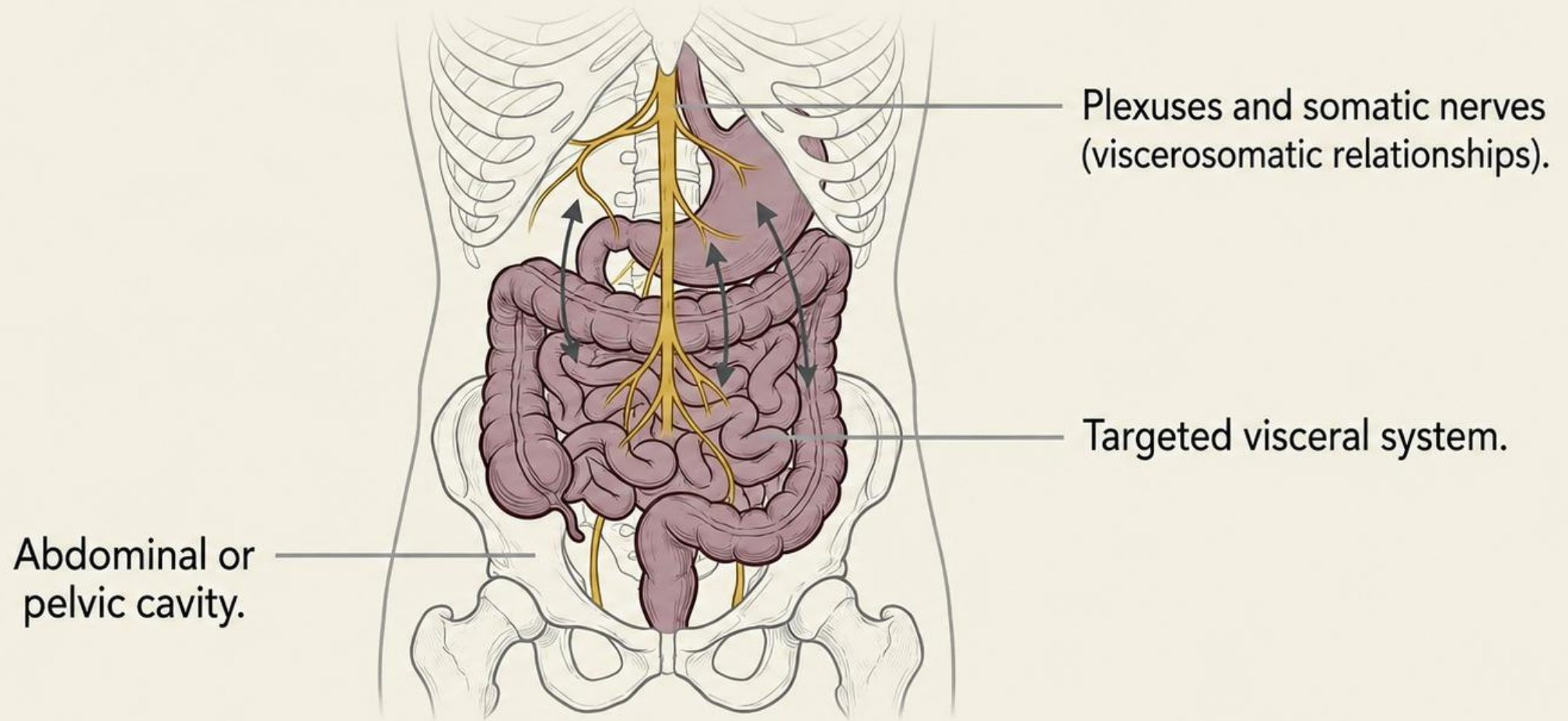
Sympathetic Innervation: Vertebral column, laterovertebral ganglionic chain.

Autonomic Nervous System: Vagus nerve (foramen magnum/jugular foramen, esophageal hiatus), pelvic parasympathetic system.



Target II: Locoregional Syndrome

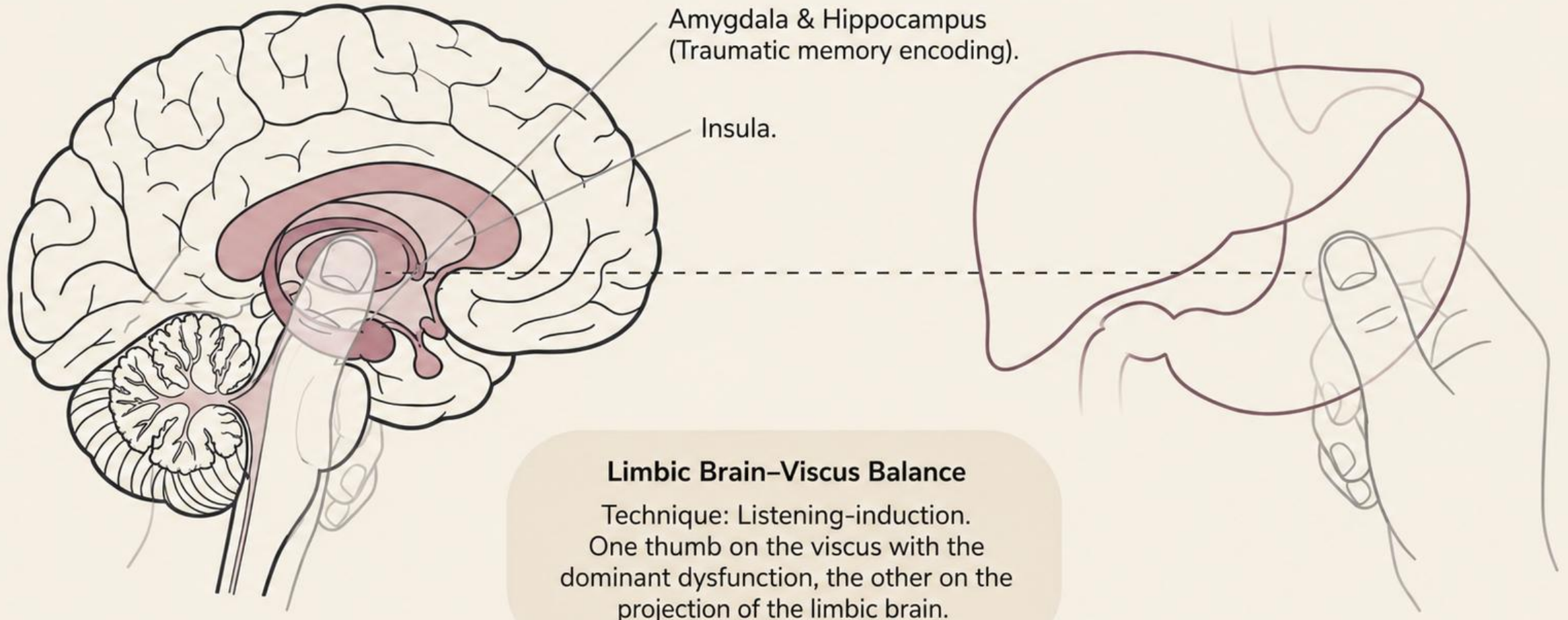
Treatment of loops related to the symptomatic region.



The objective is to release local restrictions in the involved cavity in order to restore tissue mobility and interrupt the autonomic neurovegetative loop maintained by the dysfunction.

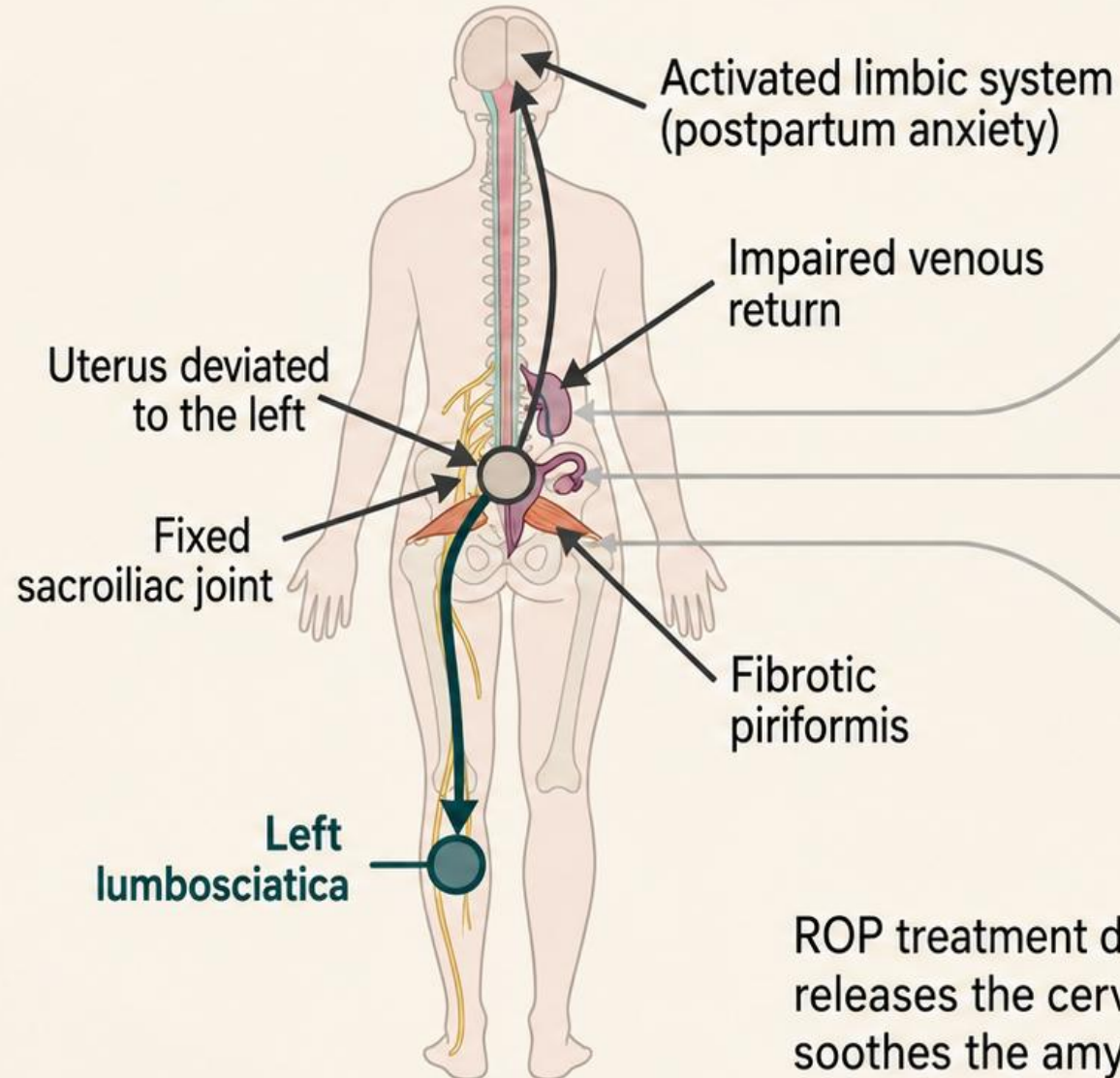
Target III: Limbic System

Integration of somatic memory.



Clinical Application: Postpartum Lumbosciatica

Primiparous patient, pain exacerbated during menstruation, prolonged labor.



Locoregional: Deviated uterus pulling on the bladder and sacrum -> piriformis fibrosis -> irritation of the sciatic nerve.

General (SGA): Sacrococcygeal fixation tensioning the dura mater (PRM). Impaired pelvic venous return.

Limbic: Anxiety related to pain and to the new maternal role.

ROP treatment does not target only the sciatic nerve, but also releases the cervico-isthmic region, reactivates the PRM, and soothes the amygdala.

Targeted ROP Treatment Plan for Patient X

Pillar 1: General Syndrome — Foundation

- Primary Respiratory Mechanism (PRM)
- Diencephalon & Brainstem
- Hormonal Axis: Pituitary Gland, Adrenal Glands
- Venous Return: Liver, Left Kidney
- Spinal Column

Pillar 2: Locoregional Syndrome — Mechanics

- Cervico-isthmic Region of the Uterus
- Pelvic Joints (Sacroiliac, Sacrococcygeal)
- Piriformis Muscle & Sacrotuberous/Sacrospinous Ligaments
- Pudendal Nerve & Left Sciatic Plexus

Pillar 3: Limbic System — Integration

- Amygdala
- Hippocampus
- Insula

ROP Treatment Plan (Case Application: Mrs. X)

Visceral & Vascular

Thorax/Abdomen: Adrenal glands, vertebral column, liver, left kidney.

General & Limbic Syndrome

Skull: MRP, diencephalon, brainstem, pituitary gland.

Limbic system: amygdala, hippocampus, insula.

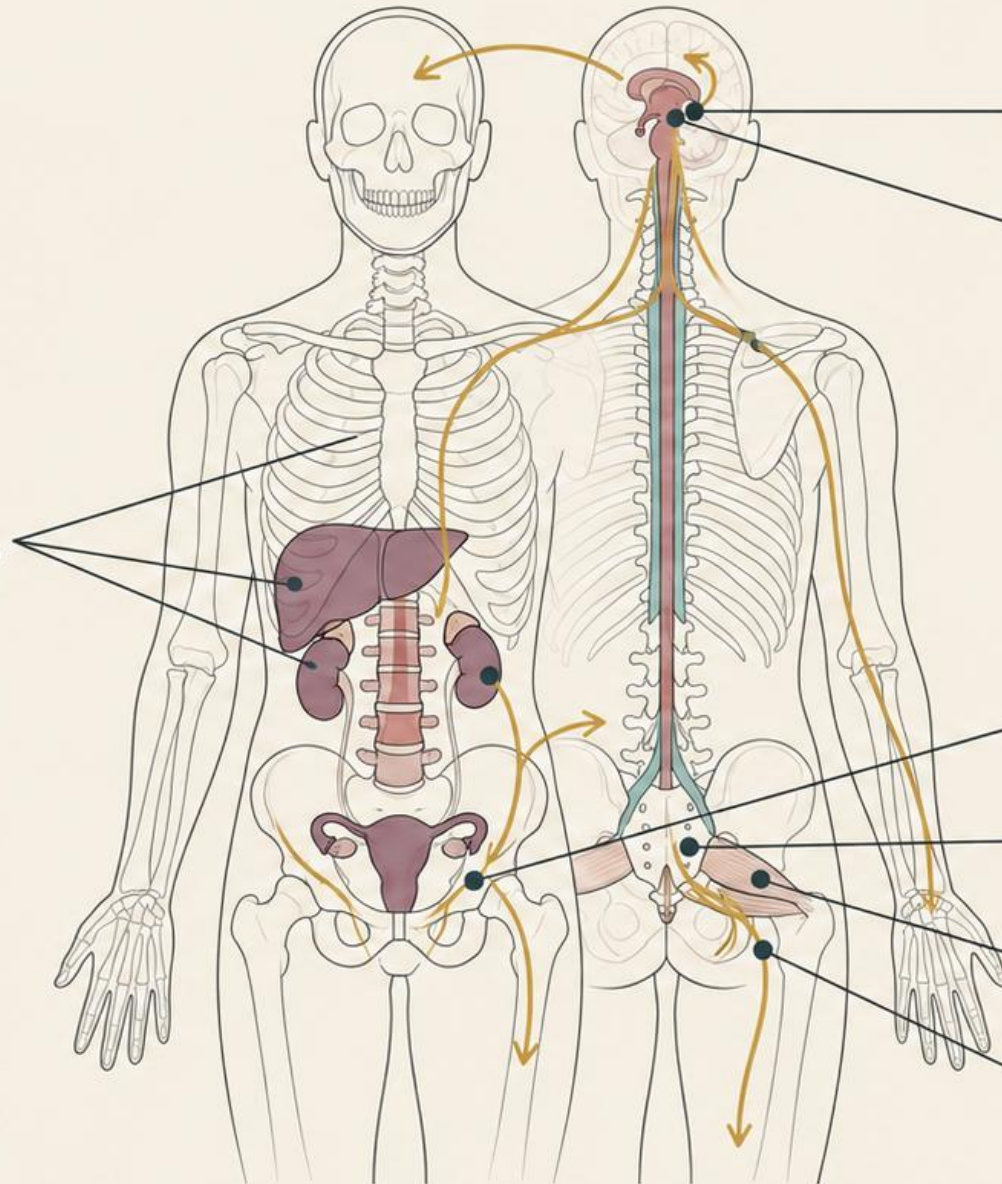
Locoregional Syndrome

Pelvis: Cervico-isthmic region of the uterus.

Joints: Sacroiliac, sacrococcygeal.

Muscles/Ligaments: Piriformis, sacrotuberous and sacrospinous ligaments.

Nerves: Pudendal nerve, sacral plexus, left sciatic nerve.



Therapeutic indications: The domain of reversible functional disorders

ROP is particularly effective in imbalances of the vasculo-neural system arising upstream of disease, where anatomical tissue alteration has not yet begun.



Musculoskeletal pain: Low back pain, neck pain, chronic tension without major structural involvement.



Post-traumatic sequelae: Osteo-musculo-articular disorders, or reversible involvement of the spinal and cranial nerves.



Visceral dysfunctions: Autonomic neurovegetative loops and tissue fixations impairing organ motility.



Somatovisceral disorders: Referred pain between the soma and the metameric visceral cavities.



Neurovegetative terrain: Emotional stress, anxiety, and dysregulation of the General Adaptation Syndrome.

Clinical Governance: Safety and Limits

✓ Indications (Functional Care)	+ Red Flags (Medical Referral Required)
Reversible and extracellular pathologies.	Non-mechanical pain, worse at night.
Musculoskeletal pain and sequelae of trauma.	Atypical thoracic pain, dyspnea.
Visceral dysfunctions and neurovegetative imbalances.	Blood in urine/stool, fever, unexplained weight loss.
Emotional stress and anxiety.	Neurological deficit (sphincter or sensory disturbances).
Integrative support (e.g., limitation of post-surgical iatrogenic effects).	

Primum non nocere. Any suspicion of organic pathology requires medical reorientation.

Global Patient Support



Rest and Recovery (ROP):
Avoid physical overexertion
during the day.



Mental Well-being: Become an
active participant in your health,
cultivate positive thoughts,
nurture emotional well-being.

*“Before you attempt to
cure someone, ask him
if he is willing to give up
the things that made him sick.”
— Hippocrates*



Lifestyle Hygiene: Healthy
nutrition, adequate hydration,
regular physical activity.



Self-Monitoring: Track your
progress for your next appointment
(Pain 0-10, sleep, transit,
energy). Objective tracking guides
future care decisions.